illness was 23 ± 4.6 years. The mean number of hospitalizations per patient was 2 (ranging from 0 to 12), with an average hospitalization duration of 72.69 days. The mean total TDAS score was 6.28 ± 4.2, the mean total PANSS score was 76.47 ± 37.24, the mean positive PANSS score was 13.84 ± 6.76, the mean negative PANSS score was 18.44 ± 11.36, and the mean general psychopathology PANSS score was 44.19 ± 25.29.
The total TDAS score was significantly higher in patients with low socioeconomic status ($p = 0.004$) and those unemployed ($p = 0.003$). We found a positive correlation between the total TDAS score and positive PANSS scores ($p = 0.001$; $r = 0.706$), general psychopathology PANSS scores ($p = 0.001$; $r = 0.577$), and total PANSS scores ($p = 0.001$; $r = 0.662$).
Conclusions: Our study shows that death anxiety among patients with schizophrenia seems to be linked to psychosocial vulnerability factors and influenced by psychotic symptoms. It is essential to better understand the nature of this anxiety, taking into account the associated factors, to ensure appropriate management.

Disclosure of Interest: None Declared

EPP102

Long-acting injectable antipsychotics and long-term functionality in schizophrenia

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Introduction: Functional recovery is recognised as the most important target in schizophrenia, complementing symptomatic remission and relapse prevention [1]. Achieving and sustaining functionality remains challenging, as only a proportion of patients reach long-term stability.
The pharmacological strategies that support sustained functionality in real-world clinical practice remain insufficiently described. Long-acting injectables (LAIs), plays a central role in relapse prevention and illness stabilisation. While LAIs are often recommended for patients with poor adherence [2], recent evidence suggests they may also contribute to improved real-life outcomes in broader patient populations [3]. Understanding treatment patterns in patients who achieve high levels of functionality may provide important insights into the determinants of recovery.
Objectives: To describe demographic and clinical characteristics, as well as antipsychotic treatment patterns, in a cohort of patients with schizophrenia identified by psychiatrists as achieving sustained functionality.
Methods: Twenty certified psychiatrists in Romania each reported on five of their most functional patients with schizophrenia, resulting in a cohort of 100 individuals (50 men, 50 women). Functionality was determined by psychiatrists' clinical judgement, supported by rating scales such as the Global Assessment of Functioning and the Social and Occupational Functioning Assessment Scale. Demographic characteristics, illness course, and psychopharmacological treatments were recorded and analysed. Ethical approval was obtained in accordance with institutional requirements.
Results: The cohort consisted of patients selected as the five most functionally stable cases from each participating psychiatrist. The mean age was 43.2 ± 8.1 years, the mean age at illness onset was 26.7 ± 5.9 years, and the mean illness duration was 16.5 ± 7.4 years. The mean duration of remission was 7.2 ± 4.4 years.
Long-acting injectable (LAI) antipsychotics were prescribed in 72% of cases, with aripiprazole LAI most frequently used (42%), followed by paliperidone LAI (14%) and risperidone LAI (13%).
Among oral agents, olanzapine (14%), aripiprazole (8%), and clozapine (8%) were most represented, with smaller proportions on quetiapine, risperidone, cariprazine, and amisulpride. Combination antipsychotic treatment was reported in 10 patients.
Conclusions: This study suggests that functionality in schizophrenia is frequently associated with prolonged remission, stable illness trajectories, and widespread use of LAI antipsychotics.
Aripiprazole LAI was the most frequently reported treatment. Previous studies have described benefits of aripiprazole LAI regarding tolerability, relapse prevention, and quality of life outcomes [4]. These results appear in line with previous evidence, while also possibly reflecting prescribing patterns or clinical selection.

Disclosure of Interest: None Declared

EPP103

Collaboration and co-design between research experts and experts with lived experience in the TRUSTING project for AI-based relapse risk assessment for individuals with psychosis

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Introduction: Healthcare systems often struggle to adequately address the needs of people with mental health conditions, leading to experiences of stigmatization, discrimination, and inadequate care. These shortcomings can deter individuals from seeking or engaging with treatment, particularly when interventions are developed without meaningful user involvement. Digital tools must be held to an especially high standard in this context, as they raise additional concerns around data privacy, security, and misuse of sensitive data. Still, digital mental health holds significant therapeutic potential, and research shows that individuals with mental health conditions view AI-based tools favorably when privacy concerns are adequately addressed.
Objectives: The TRUSTING project – a prospective multicenter study developing AI-based relapse risk assessment through language analysis in individuals with psychosis – addresses these challenges by including the perspectives of experts with lived experience.
Methods: Crucial technical components such as the TRUSTING app for data collection were co-designed in the project. A dedicated advisory user board, established by GAMIAN-Europe, provided continuous input on technical development to UiT – The Arctic University of Norway. In addition, several rounds of surveys among people with lived experience asked about their opinion on digital health and mental health care.

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Results: In our presentation, we will discuss the collaborative design process, analyzing both successful strategies and challenges. Moreover, we will present preliminary findings from the surveys, highlighting service user needs and key factors influencing engagement with digital mental health solutions.
Conclusions: The TRUSTING project illustrates how collaborative design with service users can address barriers in digital mental health and enhance the relevance of emerging AI-based tools.

Disclosure of Interest: None Declared

EPP104

The number of hospitalisations is associated with immune parameters in patients with schizophrenia in remission.

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Introduction: In the population of patients with schizophrenia, the number of psychiatric hospitalisations can serve as a proxy for cumulative disease severity or recurrence of psychotic episodes, though it may also be influenced by other factors, e.g. substance abuse or psychosocial instability. Meta-analytical studies have consistently demonstrated abnormalities in immune markers, including routine blood morphology, in individuals with schizophrenia. The assessment of these markers in relation to the number of psychiatric hospitalisations may provide valuable insights into immune-related mechanisms contributing to disease burden.
Objectives: The aim of this study was to investigate the relationship between clinical history parameters and levels of blood components, including blood morphology, lipid profile, hormones, and inflammatory markers, in patients with schizophrenia.
Methods: Patients with diagnosed schizophrenia in remission (n=37) had their blood taken in the morning. A set of tests, including blood morphology evaluation, lipid profile, C-reactive protein (CRP), free triiodothyronine (FT3) and dehydroepiandosterone sulfate (DHEA-S), was performed in the diagnostic laboratory of the University Hospital in Krakow. Each patient underwent a detailed psychiatric interview, with additional information provided by a close relative. The results were subjected to statistical analyses, using Spearman's rank correlation coefficient.
Results: Significant positive correlations between the number of hospitalisations and white blood cells (WBC) ($r = 0.42$; $p < 0.01$), neutrophils ($r = 0.34$; $p < 0.05$), lymphocytes ($r = 0.423$; $p < 0.01$), eosinophils ($r = 0.33$; $p < 0.05$), reactivated lymphocytes ($r = 0.39$; $p < 0.05$) and CRP ($r = 0.48$; $p < 0.01$) were observed. The number of hospitalisations correlated negatively with the monocyte's percentage related to WBC ($r = -0.54$; $p = 0.001$), FT3 ($r = -0.45$; $p < 0.01$), DHEA-S ($r = -0.39$; $p < 0.05$) and HDL ($r = -0.37$; $p < 0.05$). Importantly, no significant correlations were found between the blood parameters mentioned above and other indicators of the clinical picture, such as the duration of treatment, age of onset, frequency of hospitalisations or daily neuroleptic dosage equivalent.
Conclusions: The number of psychiatric hospitalisations in schizophrenia is linked to various blood parameters, mainly immune parameters. Since the same parameters do not significantly associate with the age of onset, frequency of hospitalisations, or daily antipsychotic dosage, it is plausible that observed changes reflect trait-related immune dysregulation rather than effects of medication or illness chronicity. The findings suggest an association between the number of hospitalisations and immune parameters, although the direction of this relationship remains unclear. Further research concentrated on the distinctive aspects of psychiatric hospitalisation in schizophrenia could provide a deeper understanding of the disease.

Disclosure of Interest: None Declared

EPP105

Exploring the link between Thought Disorders and Suicide Risk: evidence from a clinical cohort of adolescents

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Introduction: Thought disorders in adolescence are associated with significant distress, functional impairment, and increased vulnerability to suicide risk. Similarly, suicide risk is a major public health concern among adolescents. Although evidence suggests an association between suicidal risk and thought disorders in adolescents, the literature on this topic remains limited.
Objectives: This study aimed to characterize the presence and features of thought disorders in a clinical cohort of adolescents, stratified by suicidal risk. We sought to identify differences in vulnerability across different clinical populations (no suicidal concern, low suicidal risk, high suicidal risk) to better understand patterns of susceptibility.
Methods: 112 adolescents (12–18 years), admitted to a Child and Adolescent Neuropsychiatry unit (2020-2024), were recruited and underwent comprehensive psychodiagnostic assessments. The Columbia Suicide Severity Rating Scale (C-SSRS) was used to classify patients into no suicidal concern (NSC) and suicidal concern (SC; divideded into low vs high suicide risk), while the Comprehensive Assessment of At-Risk Mental States (CAARMS) assessed the presence and characteristics of thought disorders. Sociodemographic data, social functioning (SOFAS), and clinical severity (CGI-S) were also collected.
Results: The sample was predominantly female (83%), with a mean age of 15.5. Thought disorders were more frequent and severe in patients with SC compared to NSC. Among patients with psychosis or attenuated psychosis, SC was significantly associated with greater severity of formal thought disorders. Even among those without formal thought disorders, SC corresponded to a higher